



Beverly Oaks Animal Hospital
14302 Ventura Blvd
Sherman Oaks, CA 91423-2753
P: (818) 788-2022 / F: (818) 788-5246
info@beverlyoaksvet.com
http://www.beverlyoaksvet.com

Patient Information

Name:	Hanon
Breed (Species):	Bernedoodle (Canine)
Age (Birthday):	9 years 3 months 3 weeks (Mar 21, 2017)
Sex:	Male (neutered)
Color:	sable
Chip:	
Weight:	84.6 lbs (38.37 kg)

Owner Information

<u>Name</u>	<u>Address</u>	<u>Mobile</u>	<u>Home</u>	<u>Email</u>
brandon rose	4829 Lemona Ave, Sherman Oaks, CA 91403	323-533-0117		brandonrose2@gmail.com

Reminders

<u>Item</u>	<u>Given</u>	<u>Due</u>
KeyScreen GI Parasite PCR (antech)	Jul 13, 2026	Aug 13, 2026
OFFICE VISIT	Jul 13, 2026	Jul 13, 2027

Vitals (Jul 01, 2026 - Jul 15, 2026)

<u>Date</u>	<u>Weight</u>	<u>Heart Rate</u>	<u>Temperature</u>	<u>Respiration</u>
Jul 14, 2026	84.6 Lbs (38.37 kg) (09:29 AM)			
Jul 13, 2026	85.2 Lbs (38.65 kg) (09:28 AM)	120 bpm (07:31 PM)	101.3 F (38.5 C) (09:28 AM)	

History (Jul 01, 2026 - Jul 15, 2026)

JUL 15, 2026

Email with brandon rose
Dear brandon,
Below is a link to Hanon's lab result. If you have any questions, please call us at (818) 788-2022 or email us at info@beverlyoaksvet.com.
Thank you,
Beverly Oaks Animal Hospital

[Click here to view lab results](#)

JUL 14, 2026

 Medical Note

Complaint: oncura drugs

Plan

Notes: SWO told him everything went well going the ultrasound and that he could plan to p/u 4:3-5pm he doesn't get off work until 6pm so he will come after that.
report usually take 24-48 hrs so to keep an eye out for that email.
O said that pet was doing better this morning than he was yesterday.
Sedated with Gaba 300 mg - 2 caps @10:20am
gave torb 2mL and midaz 1.8mL IV some did go SQ but mostly all IV @12:20pm

Ultrasound report:

Case: 572827 -----Completed On: 07-15-2026 109389 ---- Hanon Rose
Reason for exam: Abdominal mass seen on radiographs

Ultrasound report:

Urinary Bladder: Urinary bladder is mildly distended with anechoic urine. Bladder wall is normal. Possible opacity at the bladder trigone. No calculi seen.

Reproductive Tract: Prostate not well visualized.

Left Kidney: Normal size with normal shape and architecture. Normal corticomedullary distinction. No pyelectasia, ureteral dilation or nephrolithiasis. The left kidney measures 6.46 cm.

Left Adrenal: Not well visualized.

Spleen: Normal in size, shape, margination and echogenicity. No masses are seen. Normal blood flow seen at hilus with color flow Doppler imaging.

Liver: Normal size and shape with smooth lobar margins. The parenchyma has normal echogenicity with normal echotexture. No focal lesions are seen. Intrahepatic bile ducts are normal. Normal vascular pattern.

Gallbladder: Gallbladder normal in size with anechoic bile and a mild amount of sludge; gallbladder wall normal. Common bile duct with no obvious dilation.

Pancreas: Mass obscures evaluation. No obvious abnormalities seen.

Right Kidney: Normal size with normal shape and architecture. Normal corticomedullary distinction. No pyelectasia, ureteral dilation or nephrolithiasis. The right kidney measures 6.90 cm.

Right Adrenal: Not well visualized.

Gastrointestinal Tract: Stomach mildly gas distended with normal wall layering and normal thickness measuring 3.1 mm. Duodenum has normal wall layering with normal thickness measuring 4.0 mm. Cranial jejunum has normal wall layering with normal thickness measuring 3.0 mm. Mid jejunum has normal wall layering with normal thickness measuring 3.0 mm. Caudal jejunum has normal wall layering with normal thickness measuring 3.2 mm. No obstructive pattern or foreign material. Colon wall is thin and uniform.

Abdominal lymph nodes: No enlargement noted.

Iliac region: No abnormalities noted.

Presence of Fluid: None.

Other: Atypical hyperechoic mass in the right cranial abdomen measuring 9.0 x 7.5 cm containing an irregular-shaped pocket of fluid.

Conclusions:

Findings:

Right cranial abdominal mass with irregular fluid pocket. Rule out abscess vs malignancy such as a carcinoma or sarcoma vs granuloma vs other. Rule out right liver vs pancreas vs mesentery vs other.

Possible bladder trigone opacity. Rule out debris, artifact, nodule, other.

Recommendations:

Exploratory surgery for mass removal with histopathology.

Abdominal CT scan could also be considered for further evaluation of the mass.

3-view thoracic radiographs to assess for pulmonary metastatic disease.

Obtain additional images of the bladder trigone with sonographer assistance, correlate with clinical concern. L.Leach, DVM

Complaint: oncura appt

Plan

Notes: Signalment: Male neutered canine ~9y

Presenting Complaint: Hanon rose presents for abdominal ultrasound following radiographic identification of a possible abdominal mass or mass effect

Patient History:

- Seen by Dr. Laurel A Leach yesterday
- current meds: Metronidazole 500mg 1po bid x5 / Cerenia 60mg 1.5 tabs po q24h / Rimadyl 75mg 1 po bid.
- Radiographs from yesterday's visit revealed a possible abdominal mass or mass effect in the mid-abdomen with intestinal displacement to one side
- Client reports weakness or pain in the left forelimb

Vital Signs:

Mentation: BAR
Weight: 84.6 lbs
Temperature: WNL
Heart Rate: 150 bpm
Respiratory Rate: WNL
Mucous Membranes: Pink and moist <2
BCS: 5/9

Findings:

Eyes: Mild lens opacity OU, no discharge or redness
Ears: Clean AU, no discharge or inflammation
Nose: Clean, no discharge
Oral: Moderate tartar noted
Heart: Murmur assessment limited due to marked panting; rhythm regular
Lungs: Patient panting throughout exam; lung fields clear bilaterally, no crackles or wheezes
Musculoskeletal: Weakness/pain evident in left forelimb; ambulatory x4
Integument: Curly hair coat, no ectoparasites
Peripheral Lymph Nodes: Non-palpable or less than 0.5 cm
Abdomen: Semi-tense; non-painful, no masses appreciated on palpation
Urogenital: Normal external genitalia, no discharge or inflammation
Rectal Exam: Perianal area clean
Neurologic: Full neuro exam not performed. Neurologic status appears WNL

Problem List:

- Possible abdominal mass/mass effect (per radiograph) - r/o neoplasia, organomegaly, abscess, lymphadenopathy
- Left forelimb lameness/weakness - r/o orthopedic disease, neurologic disease, soft tissue injury
- Elevated liver enzymes (AST, ALP) - r/o hepatopathy, neoplasia, hyperadrenocorticism, infectious disease
- Proteinuria (3+) with hematuria - r/o glomerulonephropathy, UTI, neoplasia
- Hypercholesterolemia - r/o hyperadrenocorticism, hypothyroidism, hepatic disease
- Elevated sodium:potassium ratio - r/o hyperadrenocorticism, lab variation
- Elevated globulin / decreased A:G ratio - r/o chronic inflammation, infectious disease, neoplasia
- Hypophosphatemia - r/o nutritional, renal loss, hyperparathyroidism
- Elevated Precision PSL - r/o hyperadrenocorticism
- Neutrophilia - r/o stress leukogram, infection, inflammation
- Lens opacity OU - r/o nuclear sclerosis, cataract

Diagnostics:

- Abdominal ultrasound: scheduled today (sedation planned prior to procedure)
- Blood work from prior visit (2026-07-13), reviewed at this visit:
- Globulin: 3.8 g/dL (H) [ref: 1.6–3.6]

- Albumin:globulin ratio: 0.7 (L) [mildly decreased]
- AST: 110 (H) [ref: 15-66]
- Alkaline phosphatase: 157 (H) [ref: 5-131]
- Phosphorus: 1.4 mg/dL (L) [ref: 2.5-6.0]
- Na:K ratio: 39 (H) mild
- Cholesterol: 330 mg/dL (H) [ref: 92-324]
- Precision PSL: 182 (H)
- Neutrophils: 10,692 (H)
- Urinalysis: USG >1.060, protein 3+, bilirubin 2+, occult blood 2+, RBCs present
- Fecal: no parasites or ova observed

Treatment Plan/Medications:

- Sedation to be administered prior to abdominal ultrasound
- Gabapentin 300mg 2 caps po 2h prior.
- Butorphanol (10mg/ml) 2ml IV
- midazolam (5mg/ml) 1.8ml IV

- Abdominal ultrasound being performed today to further evaluate radiographic finding of possible abdominal mass

J.Jellinek, DVM

 1 Unit(s) of Injection-Sedation (81-150lbs).

 1 Unit(s) of HAZARDOUS WASTE DISPOSAL FEE.

 Oncura Abdominal Protocol .

JUL 13, 2026

 09:30 AM. Visit with DVM Laurie Leach for panting, stomach pain/poss limping.

Medical Note

Complaint: panting, stomach pain/poss limping

Provider: DVM Laurie Leach

Subjective

Notes: Signalment: Adult male canine

Presenting Complaint: Hanon presents for panting, abdominal pain, and left forelimb lameness

Same thing that was going on 5 weeks ago when he first came in

Vomited a couple of times yesterday- not vomiting Saturday. Sunday: intact dog food immed after he ate, owner thinks, he had small corner of the bread last night.

has a good appetite; food was offered but pet didnt eat it because pet eats when owner eats.

Hiding underneath the Piano, which is a symptom to the owner that there is something wrong.

Lays with his neck extended

Abdomen is tight and swollen

O. feels lumps that he thinks maybe associated with the pet feeling poorly as owner doesnt feel them when pet feeling healthy.

Been limping on left side -- o had video that owner showed to Dr. Wood. Expl. we cant keep those in the practice management

Patient History:

- No additional history provided

Objective

Vital Signs:

Weight: 85.2 Lbs.

Temperature: 101.3 F.

Heart rate: 120 bpm.

Notes: Vital Signs:

Weight: 82.5

Temperature: WNL

Heart Rate: 120 bpm

Respiratory Rate: ~100 breaths/min

Mucous Membranes: Pink and moist

BCS: 4/9; Muscle condition: WSAVA reduced

Findings:

Eyes: Clear OU, no discharge or redness, mild corneal haze OU, anterior chamber clear, pupil size even, adnexa unrem

Ears: Clean AU, no discharge or inflammation, horizontal canal clear and tympanum intact AU

Nose: Clean, no discharge, good apparent airflow thru nostrils

Oral: Heavy tartar; gingiva pink; possible epulis on distal aspect of 204; oral cavity otherwise unremarkable

Heart: Normal rate and rhythm, no murmurs or arrhythmias

Pulses: Strong and synchronous

Lungs: Clear lung fields bilaterally, no crackles or wheezes, rate and depth of respiration unrem

Musculoskeletal:

FORELEGS: Left forelimb lameness confirmed on observation; reluctant to bear weight on left fore; trembling; muscle mass loss with pain on palpation of left mid-shaft humerus/triceps region-- but maybe it is medial and in the region of the brachial plexus or nerves going to the foreleg? no masses palpable but pet definitely trembles when this area palpated; no pain on elbow extension; full carpus flexion; spine of scapula palpates normally; musculature over scapula unremarkable; right forelimb musculature WNL;

HINDLEGS: stifles palpate normally bilaterally; hocks palpate normally; caudal hyperextension unremarkable

Integument: Fecal soiling of perineum--pet has diarrhea and owners observation of pet straining may relate to dyschezia; healthy hair coat and skin, no ectoparasites seen. right dorsolateral abdominal wall suspect large lipoma. Left ventrolateral thorax has smaller, ~1 inch softly palpable mass

Peripheral Lymph Nodes: Non-palpable or less than 0.5 cm

Abdomen: Soft, non-tender on palpation, no masses or organomegaly on palpation but radiographs show mass effect.

Urogenital: Normal external genitalia, no discharge or inflammation

Rectal Exam: Not performed

Neurologic: Full neuro exam not performed. Neurologic status appears WNL

Assessment

Diagnoses:

Notes: Problem List:

- Left forelimb lameness with muscle mass loss - r/o soft tissue injury (triceps/mid-shaft humerus), fracture, neoplasia, osteoarthritis
- Tachypnea - r/o pain response, anxiety, primary respiratory disease
- Dental disease - r/o periodontal disease, epulis (204)
- Perineal fecal soiling/diarrhea - r/o dietary indiscretion, infectious gastroenteritis, IBD

Plan

Notes: Diagnostics:

- Radiographs performed (via Antech Imaging):
- Left forelimb (multiple views): cortical bone thickness WNL throughout; equivocal mild irregularity at shoulder joint (soft finding, not definitive); elbow WNL; no arthritis noted at shoulder
- Abdomen (multiple views): mid-abdominal mass effect, estimated ~3.7 x 5 inches, displacing intestines laterally; significant colonic gas; no evidence of GDV; stomach WNL; bladder small; kidneys unremarkable
- SA705 submitted to Antech: pending results not back by 6 pm when I spoke with owner
- T991 submitted to Antech: results pending

Treatment Plan/Medications:

- Perineum cleaned to prevent perianal dermatitis
- Rx: metronidazole (Flagyl) 500 mg - 1 PO BID x 5 days (GI antibiotic for diarrhea)
- Rx: maropitant (anti-emetic; secondary benefit of pain/discomfort relief) 60 mg #6 1.5 PO BID SID
- Rx: NSAID (pain management for left forelimb lameness) - Carprofen 75mg - 1 PO BID 7 days #14

Client Communication:

- Radiographs reviewed with client:
 - Left forelimb: no fracture, no arthritis identified; etiology of lameness remains unclear; possible bicipital tenosynovitis or soft tissue/ligamentous injury discussed
 - Abdomen: mid-abdominal mass confirmed; GDV ruled out
 - Subcutaneous soft masses noted on exam and dorsal fat visible on radiograph; suspected lipomas, aspiration not performed
 - Discussed three options for abdominal mass workup:
 - Abdominal ultrasound (~\$600-700): first-line; allows visualization of structure, possible guided sampling
 - CT scan (~\$3,000): gold standard for surgical planning; best resolution of adjacent structures and vascular supply; recommended prior to specialist referral
 - Exploratory surgery (\$2,000-4,000 estimated): risk of finding mass adherent to multiple organs requiring specialist; may need to close and refer regardless
 - Client elected to proceed with abdominal ultrasound; scheduled for tomorrow
 - Discussed possibility of specialist surgical referral depending on ultrasound findings; specialists typically require CT scan prior to surgery
 - Current medications address symptomatic relief; further workup needed to address primary abdominal mass
- L.Leach, DVM

 RADIOGRAPH-ADDITIONAL (EACH).

 RADIOGRAPH - LEG.

 1 Unit(s) of OFFICE VISIT.

 10 Tablet(s) of Metronidazole (flagyl) 500mg Tab.
admin 1 tablet by mouth every 12 hours for 5 days for diarrhea.

 6 Unit(s) of Maropitant Citrate 60 mg per tab.
admin 1 and 1/2 tab by mouth once a day for 3 days, then if and as needed.

 14 Unit(s) of Caprievie Caplet 75mg.
Admin 1 tablet by mouth every 12 hours WITH FOOD. This is a non steroidal anti inflammatory and pain relieving medication

 **Items:** Senior Wellness Profile 1 (Superchem, CBC, T4, UA), KeyScreen GI Parasite PCR (antech)

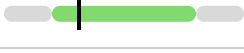
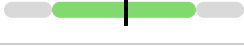
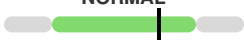
Ordered: Jul 13, 2026

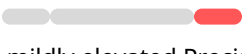
Received: Jul 14, 2026

Source: ANTECH WEST

Results: SuperChem

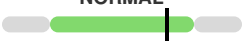
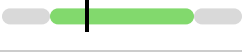
<u>Name</u>	<u>Result</u>	<u>Reference range</u>	
TOTAL PROTEIN	6.6	5.0-7.4 g/dL	NORMAL

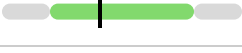
ALBUMIN	2.8	2.7-4.4 g/dL	
GLOBULIN	3.8 (H)	1.6-3.6 g/dL	
A/G RATIO	0.7 (L)	0.8-2.0	
AST (SGOT)	110 (H)	15-66 IU/L	
ALT (SGPT)	47	12-118 IU/L	
ALK PHOS	157 (H)	5-131 IU/L	
GGT	3	1-12 IU/L	
T. BILIRUBIN	0.2	0.1-0.3 mg/dL	
BUN	16	6-31 mg/dL	
CREATININE	1.1	0.5-1.6 mg/dL	
SDMA	10.9	<14.0 UG/dL	
BUN/CREAT RATIO	15	4-27	
PHOSPHORUS	1.4 (L)	2.5-6.0 mg/dL	
GLUCOSE	121	70-138 mg/dL	
CALCIUM	10.5	8.9-11.4 mg/dL	
CORRECTED CALCIUM	11.2		
MAGNESIUM	1.6	1.5-2.5 mEq/L	
SODIUM	150	139-154 mEq/L	

POTASSIUM	3.8	3.6-5.5 mEq/L	
NA/K RATIO	39 (H)	27-38	
CHLORIDE	115	102-120 mEq/L	
CHOLESTEROL	330 (H)	92-324 mg/dL	
TRIGLYCERIDE	146	29-291 mg/dL	
AMYLASE	580	290-1125 IU/L	
PrecisionPSL	182 (H)	24-140 U/L	
In dogs with appropriate clinical signs, this mildly elevated PrecisionPSL result is equivocal for a diagnosis of pancreatitis. In dogs without clinical signs of pancreatitis, a mild PrecisionPSL elevation is an insignificant finding.			

CPK	491	59-895 IU/L	
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Results: Complete Blood Count

<u>Name</u>	<u>Result</u>	<u>Reference range</u>	
WBC	13.2	4.0-15.5 10 ³ /mL	
RBC	6.7	4.8-9.3 10 ⁶ /mL	
HGB	16.2	12.1-20.3 g/dL	
HCT	50	36-60 %	
MCV	75	58-79 fL	
MCH	24.3	19-28 pg	
MCHC	32	30-38 g/dL	

Platelet Count	217	170-400 10 ³ /mL	
Platelet Estimate	Adequate		
Neutrophils	81 (H)	60-77 %	
Bands	0	0-3 %	
Lymphocytes	15	12-30 %	
Monocytes	3	3-10 %	
Eosinophils	1 (L)	2-10 %	
Basophils	0	0-1 %	
Absolute Neutrophils	10692 (H)	2060-10600 /mL	
Absolute Lymphocytes	1980	690-4500 /mL	
Absolute Monocytes	396	0-840 /mL	
Absolute Eosinophils	132	0-1200 /mL	
Absolute Basophils	0	0-150 /mL	

Results: T4

<u>Name</u>	<u>Result</u>	<u>Reference range</u>
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T4

2.2

0.8-3.5 mg/dL

NORMAL



Diagnosis of canine hypothyroidism: for dogs with consistent clinical signs and without non-thyroidal illness, a low-to-low normal T4 concentration (e.g., <1.0 ug/dL) could support hypothyroidism. Assessment of free T4 by ED, TSH and possibly thyroglobulin autoantibodies (TGAA) can aid in the diagnosis. Canine hypothyroidism and synthetic T4 titration: Sample at 4 to 6 hours after dosing and aim for T4 concentrations in the mid to just above the upper end of the reference interval. Dose reduction should be considered when T4 concentration is greater than approximately 6.0 ug/dL. Always interpret results alongside the patient's clinical signs and response to treatment.

Results: Urinalysis-Complete

<u>Name</u>	<u>Result</u>	<u>Reference range</u>	
Collection Method	Not Stated		
Color	Dark Yellow		
Appearance	TURBID		
Specific Gravity	1.060 (H)	1.015-1.050	HIGH
pH	6.5	5.5-7.0	NORMAL
Protein	3+ (H)	NEGATIVE	
	Urine protein:creatinine ratio testing is recommended (if the sediment is inactive) to help determine the clinical significance of proteinuria.		
Glucose-Strip	NEGATIVE Result Verified	NEGATIVE	
Ketones	NEGATIVE Result Verified	NEGATIVE	
Bilirubin	2+ (H)	NEG TO 1+	
Occult Blood	2+ (H)	NEGATIVE	
WBC	0-1	0-3 HPF	
RBC	11-20 (H)	0-3 HPF	
Casts	NONE SEEN	Hyaline 0-3 LPF	
Calcium Oxalate Dihydrate Crystals	0-1	HPF	
Amorphous Crystals	0-1	HPF	

Bacteria	NONE SEEN	None Seen HPF
Squamous Epithelia	0-1	0-3 HPF
Fat Droplets	>50	HPF

Results: KeyScreen GI Parasite PCR Panel

<u>Name</u>	<u>Result</u>	<u>Reference range</u>
Ancylostoma spp.	Undetected	
A. caninum resistance marker	Undetected	
Uncinaria stenocephala	Undetected	
Toxocara spp.	Undetected	
Toxocara canis	Undetected	
Toxocara cati	Undetected	
Toxascaris leonina	Undetected	
Baylisascaris procyonis	Undetected	
Trichuris spp.	Undetected	
Giardia duodenalis	Undetected	
Giardia Zoonotic	Undetected	
Cryptosporidium canis	Undetected	
Cryptosporidium felis	Undetected	
Cystoisospora spp.	Undetected	
Eimeria spp.	Undetected	
Dipylidium caninum	Undetected	
Echinococcus multilocularis	Undetected	
Echinococcus granulosus	Undetected	
Taenia spp.	Undetected	
Neospora caninum	Undetected	

Comment

A DETECTED KeyScreen GI Parasite PCR result in a patient with clinical signs that are appropriate to the organism, suggests this is the likely cause of the clinical signs. In the absence of clinical signs, parasite detection could suggest a subclinical infection or be related to coprophagia. Subclinical infection may need to be treated in cases where the parasite is zoonotic, has the potential to cause clinical signs or where continued shedding contributes to environmental contamination. An UNDETECTED KeyScreen GI Parasite PCR result indicates that no parasitic organism was detected. An undetected PCR result most often indicates absence of infection but might also occur after successful treatment or with spontaneous resolution of infection. Undetected results due to cyclical shedding may be overcome with repeat testing or by testing pooled samples collected over multiple days. For infections with an extra-intestinal phase (e.g., echinococcosis, toxoplasmosis, neosporosis), an undetected KeyScreen GI Parasite PCR result does not rule out systemic infection. If systemic infection is suspected, additional diagnostic investigation is indicated. As a reference, we have provided links to CAPC guidelines. CAPC is an independent, non-profit organization. Veterinarians: If the KeyScreen GI Parasite PCR result does not explain the clinical signs or if you require additional interpretive assistance, consultation with an internist is available free of charge (Monday to Friday 8am to 8pm EST, Saturday 9am to 6pm EST) at 1-888-838-4636.

 **Items:** CYSTOCENTESIS- URINE COLLECTION

Ordered: Jul 13, 2026

 Email with brandon rose
Dear brandon,
We are looking forward to seeing you and Hanon on Jul 13, 2026 at 9:30 AM. If you need to reschedule your appointment, please call us at (818) 788-2022 or email us at info@beverlyoaksvet.com.
Thank you,
Beverly Oaks Animal Hospital

Patient:

Patient ID: 109389
First Name: Hanon
Last Name: Rose
Gender: Male, Neutered
Species: Canine
Breed: Bernadoodle
Age: 9.00
Weight (lbs): 84.60

Details:

ID: 572827
Priority: Routine
Status: Finalized
Submitted: 07-14-2026
Closed: 07-15-2026

Specialist(s):

Name: Dr. Cindy Panek, DVM,
DACVIM (SAIM)
Email: telemed@oncurepartners.com
Referred To:
Assisted By:

Services Requested:

Comprehensive Abdominal Consult (Includes US Labs History Review & Recommendations)

Clinical History

When was the last time the patient ate? Please include the approximate time and what they ate

Late yesterday afternoon

July- Complimentary Bi-cavity add-on?

No

Did you receive sonographer assistance?

No

Patient history: (Copy & Paste NOT Supported) Please include only 6 months of history and reason for this exam: Documents attached are supplemental information and do not replace history

Radiographs from yesterday's visit revealed a possible abdominal mass or mass effect in the mid-abdomen with intestinal displacement to one side

If sedation was used, please describe

Sedation to be administered prior to abdominal ultrasound- Gabapentin 300mg 2 caps po 2h prior. - Butorphanol (10mg/ml) 2ml IV- midazolam (5mg/ml) 1.8ml IV

Abnormal Lab Results. Please include abnormal lab values here performed within the last 6 months. (Please limit attachment to the most current lab work)

Blood work from prior visit (2026-07-13), reviewed at this visit: - Globulin: 3.8 g/dL (H) [ref: 1.6–3.6] - Albumin:globulin ratio: 0.7 (L) [mildly decreased] - AST: 110 (H) [ref: 15–66] - Alkaline phosphatase: 157 (H) [ref: 5–131] - Phosphorus: 1.4 mg/dL (L) [ref: 2.5–6.0] - Na:K ratio: 39 (H) mild - Cholesterol: 330 mg/dL (H) [ref: 92–324] - Precision PSL: 182 (H) - Neutrophils: 10,692 (H) - Urinalysis: USG 1.060, protein 3+, bilirubin 2+, occult blood +, RBCs present - Fecal: no parasites or ova observed

Current medications and dosages, please state when started:

Started 7/13/26 Metronidazole 500mg 1po bid x5 / Cerenia 60mg 1.5 tabs po q24h / Rimadyl 75mg 1 po bid.

Was patient sedated during exam?

Yes

Previous Study Completed By Oncura To Compare?

No

Interpretation

Examination performed: A complete abdominal ultrasound is submitted for review. A total of 63 still images were reviewed, and cine loops were also reviewed.

Reason for exam: Abdominal mass seen on radiographs

Ultrasound report:

Urinary Bladder: Urinary bladder is mildly distended with anechoic urine. Bladder wall is normal. Possible opacity at the bladder trigone. No calculi seen.

Reproductive Tract: Prostate not well visualized.

Left Kidney: Normal size with normal shape and architecture. Normal corticomedullary distinction. No pyelectasia, ureteral dilation or nephrolithiasis. The left kidney measures 6.46 cm.

Left Adrenal: Not well visualized.

Spleen: Normal in size, shape, margination and echogenicity. No masses are seen. Normal blood flow seen at hilus with color flow Doppler imaging.

Liver: Normal size and shape with smooth lobar margins. The parenchyma has normal echogenicity with normal echotexture. No focal lesions are seen. Intrahepatic bile ducts are normal. Normal vascular pattern.

Gallbladder: Gallbladder normal in size with anechoic bile and a mild amount of sludge; gallbladder wall normal. Common bile duct with no obvious dilation.

Pancreas: Mass obscures evaluation. No obvious abnormalities seen.

Right Kidney: Normal size with normal shape and architecture. Normal corticomedullary distinction. No pyelectasia, ureteral dilation or nephrolithiasis. The right kidney measures 6.90 cm.

Right Adrenal: Not well visualized.

Gastrointestinal Tract: Stomach mildly gas distended with normal wall layering and normal thickness measuring 3.1 mm. Duodenum has normal wall layering with normal thickness measuring 4.0 mm. Cranial jejunum has normal wall layering with normal thickness measuring 3.0 mm. Mid jejunum has normal wall layering with normal thickness measuring 3.0 mm. Caudal jejunum has normal wall layering with normal thickness measuring 3.2 mm. No obstructive pattern or foreign material. Colon wall is thin and uniform.

Abdominal lymph nodes: No enlargement noted.

Iliac region: No abnormalities noted.

Presence of Fluid: None.

Other: Atypical hyperechoic mass in the right cranial abdomen measuring 9.0 x 7.5 cm containing an irregular-shaped pocket of fluid.

Conclusions:

Findings:

Right cranial abdominal mass with irregular fluid pocket. Rule out abscess vs malignancy such as a carcinoma or sarcoma vs granuloma vs other. Rule out right liver vs pancreas vs mesentery vs other.

Possible bladder trigone opacity. Rule out debris, artifact, nodule, other.

Recommendations:

Exploratory surgery for mass removal with histopathology.

Abdominal CT scan could also be considered for further evaluation of the mass.

3-view thoracic radiographs to assess for pulmonary metastatic disease.

Obtain additional images of the bladder trigone with sonographer assistance, correlate with clinical concern.

Disclaimer:

Please note: This report was generated with AI assistance for dictation only. While we strive for accuracy, minor typographical errors may be present. Please contact us if you have any concerns about this case interpretation.

Thank you for your referral with Oncura Partners. If you have questions, concerns, or if we can be of further help with this patient please feel free to call us at 512-220-1400. Pet owners, please address any questions with your primary veterinarian.

Disclaimer

The diagnostic quality and accuracy of this report is directly dependent upon the caliber of the submitted images. The specialist is only able to report on well-defined organs, structures, and abnormalities that are fully visible in the submitted images. Not all diseases are detectable by ultrasound. Some diseases may be present without any sonographic features. Factors that frequently limit the diagnostic quality/utility of an examination may include but are not limited to sonographic artifact, patient body habitus, positioning, incomplete examinations, and technical approaches utilized by the examiner. A thorough clinical history is required for a comprehensive interpretation. Quality and accuracy of this report could be limited if lack of patient identification, date of imaging, and location of imaging is not provided.

Addendum policy: You must contact us within 24 hours of the completed consult to receive an addendum or additional charges may apply. The images do not have to be acquired within 24 hours but you do have to contact us. Addendums only apply if the specialist recommends additional ultrasound images or if you have questions that require clarification by the specialist. Addendums do not apply to follow-up radiographs even if recommended by the specialist; a separate consultation is required for follow-up radiographs.

**Beverly Oaks Animal Hospital**

14302 Ventura Blvd
Sherman Oaks, CA 91423-2753
Phone: (818) 788-2022
Fax: (818) 788-5246
info@beverlyoaksvet.com
http://www.beverlyoaksvet.com

Estimate

109389.

Client: brandon rose
Address: 4829 Lemona Ave
Sherman Oaks, CA 91403

Estimate No: 24964
Estimate Date: Jul 13, 2026
Expires: Aug 12, 2026
Terms: Due upon receipt

85.2165

Item:	Patient:	Qty:	Price:	Line Total:
OFFICE VISIT	Hanon	1 Unit(s)	\$85.00	\$85.00
RADIOGRAPH - LEG	Hanon	1 Unit(s)	\$508.54	\$508.54
RADIOGRAPH-ADDITIONAL (EACH)	Hanon	1 - 2 Unit(s)	\$143.63 - \$287.26	\$143.63 - \$287.26
☒ Senior Wellness Profile 1 (Superchem, CBC, T4, UA)	Hanon	1 Unit(s)	\$348.69	\$348.69
SAMPLE COLLECT BLOOD	Hanon	1 Unit(s)	\$29.94	\$29.94
CYSTOCENTESIS- URINE COLLECTION	Hanon	1 Unit(s)	\$25.49	\$25.49
☐ KeyScreen GI Parasite PCR (antech)	Hanon	1 Unit(s)	\$118.55	\$118.55
Notes: no poop. ↓ need o to bring sample.				
Approved By: approved verbally on phone to Sheena				
Approved On: Jul 13, 2026				
Subtotal:				\$1,259.84 - \$1,403.47
Tax (T):				\$0.00
Total:				\$1,259.84 - \$1,403.47

Please note a 75% deposit is required.

Please sign the attached care proposal information sheet. This informs our staff regarding certain provisions of the California Practice Act; guides care in special circumstances; and allows the team to move forward with the care identified in this financial estimation.

Thanks for letting us care for you and your pet!

DNR

IF MY PET'S CONDITION SHOULD DETERIORATE TO THE POINT OF CARDIAC AND/OR RESPIRATORY ARREST (**MUST CHOOSE ONE**):

_____ A) **PLEASE MAKE ALL REASONABLE ATTEMPTS TO RESUSCITATE**

_____ B) **PLEASE DO NOT RESUSCITATE**

Note: RESPIRATORY AND/OR CARDIAC ARRESTS CARRY A VERY UNFAVORABLE PROGNOSIS. ADDITIONAL COSTS ARE INCURRED TO RESUSCITATE THESE PATIENTS, AND MAY INCREASE YOUR FEES AS MUCH AS \$250 TO \$750 PER INCIDENT. IF YOU WOULD LIKE FURTHER CLARIFICATION PLEASE ASK THE ATTENDING VETERINARIAN OR TECHNICIAN.

MUTUAL AGREEMENTS

What you should expect of us:

We will always treat you and your pet with kindness and respect;

We will always recommend what we think is in your pet's best interest;

We will always be honest;

We will provide you with cost estimates prior to any testing or procedures.

What we expect of our clients:

You will treat all of our staff with respect;

You will trust that our recommendations are made in good faith in the interest of your pet;

You will let us know of any financial boundaries that you may have set, or which we cannot cross;

You will let us know if you do not understand our assessment or our treatment recommendations;

You will not let your pet suffer pain or illness without treatment.

~ Thank you.

**Beverly Oaks Animal Hospital**

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Estimate

Client: brandon rose
Address: 4829 Lemona Ave
Sherman Oaks, CA 91403

Estimate No: 24975
Estimate Date: Jul 13, 2026
Expires: Aug 12, 2026
Terms: Due upon receipt

Item:	Patient:	Qty:	Price:	Line Total:
Oncura Abdominal Protocol	Hanon	1 Unit(s)	\$590.50	\$590.50
Injection-Sedation (81-150lbs)	Hanon	1 Unit(s)	\$186.81	\$186.81
HAZARDOUS WASTE DISPOSAL FEE	Hanon	1 Unit(s)	\$5.49	\$5.49

Notes:

Subtotal:	\$782.80
Tax (T):	\$0.00
Total:	\$782.80

Please note a 75% deposit is required.

Please sign the attached care proposal information sheet. This informs our staff regarding certain provisions of the California Practice Act; guides care in special circumstances; and allows the team to move forward with the care identified in this financial estimation. Thanks for letting us care for you and your pet!

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You will not let your pet suffer pain or illness without treatment.

~ Thank you.



The Standard of
Veterinary Excellence

Beverly Oaks Animal Hospital
ANESTHESIA, SURGICAL & MEDICAL RELEASE

CLIENT NAME: Hamon Brandon Rose

PET'S NAME: Hamon BREED: Bernedoodle

AGE: 9 COLOR: Sable SEX: M

PROCEDURE(S): Ultrasound

WHEN DID YOUR PET EAT LAST: Yesterday late afternoon

PLEASE READ CAREFULLY

I, The Undersigned, Certify that I am the Owner, or Authorized Agent for the Owner, of the Animal Described Above. I Authorize the Doctor and Assistants on Duty to Perform the Procedures Listed Above and on the Attached Estimate, Including Administration of Pain Relief Medications, Sedatives and/or Anesthetics, as well as any Necessary and Appropriate Medical, Radiological, Surgical, Nursing, Diagnostic, and/or Emergency Care for the Animal. I have been Advised as to the Nature of the Procedures and the Potential Risks. I also Understand that no Guarantee of Successful Treatment Can Be Made.

I have Read and Understand the Reasons For and the Risks of the Above and Attached Authorized Procedures, and Assume Full Financial Responsibility for All Charges and Services Incurred on the Described Animal.

SIGNATURE: [Signature] DATE: 07-14-2026

PRINT NAME: Brandon Rose

PHONE NUMBER(S) WHERE WE CAN REACH YOU TODAY:

WORK: _____

HOME: _____

CELL: 323-533-0117

Dr. Laurie, LEACH

Received
07/14/2026

Reported
07/14/2026

Patient Name Hanon	Owner Rose Brandon	Species Canine	Breed Mixed	Sex CM	Age 9Y	Chart # N
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SuperChem

Test Requested	Result	Reference Interval		Visual Ref. Interval
TOTAL PROTEIN	6.6	5.0-7.4	g/dL	
ALBUMIN	2.8	2.7-4.4	g/dL	
GLOBULIN	3.8	1.6-3.6	g/dL	
A/G RATIO	0.7	0.8-2.0		
AST (SGOT)	110	15-66	IU/L	
ALT (SGPT)	47	12-118	IU/L	
ALK PHOS	157	5-131	IU/L	
GGT	3	1-12	IU/L	
T. BILIRUBIN	0.2	0.1-0.3	mg/dL	
BUN	16	6-31	mg/dL	
CREATININE	1.1	0.5-1.6	mg/dL	
SDMA	10.9	<14.0	UG/dL	
BUN/CREAT RATIO	15	4-27		
PHOSPHORUS	1.4	2.5-6.0	mg/dL	
GLUCOSE	121	70-138	mg/dL	
CALCIUM	10.5	8.9-11.4	mg/dL	
CORRECTED CALCIUM	11.2			
MAGNESIUM	1.6	1.5-2.5	mEq/L	
SODIUM	150	139-154	mEq/L	
POTASSIUM	3.8	3.6-5.5	mEq/L	
NA/K RATIO	39	27-38		
CHLORIDE	115	102-120	mEq/L	
CHOLESTEROL	330	92-324	mg/dL	
TRIGLYCERIDE	146	29-291	mg/dL	
AMYLASE	580	290-1125	IU/L	
PrecisionPSL ⓘ	182	24-140	U/L	
CPK	491	59-895	IU/L	

Complete Blood Count

Test Requested	Result	Reference Interval		Visual Ref. Interval
WBC	13.2	4.0-15.5	10 ³ /μL	
RBC	6.7	4.8-9.3	10 ⁶ /μL	
HGB	16.2	12.1-20.3	g/dL	
HCT	50	36-60	%	
MCV	75	58-79	fL	
MCH	24.3	19-28	pg	
MCHC	32	30-38	g/dL	
Platelet Count	217	170-400	10 ³ /μL	
Platelet Estimate	Adequate			
Neutrophils	81	60-77	%	
Bands	0	0-3	%	
Lymphocytes	15	12-30	%	
Monocytes	3	3-10	%	
Eosinophils	1	2-10	%	
Basophils	0	0-1	%	
Absolute Neutrophils	10692	2060-10600	/μL	

Absolute Lymphocytes	1980	690-4500	/μL	
Absolute Monocytes	396	0-840	/μL	
Absolute Eosinophils	132	0-1200	/μL	
Absolute Basophils	0	0-150	/μL	

Differential	Absolute	Reference Interval	Visual Ref. Interval
Neutrophils	10692	2060-10600	/μL
Bands	0	0-3	
Lymphocytes	1980	690-4500	/μL
Monocytes	396	0-840	/μL
Eosinophils	132	0-1200	/μL
Basophils	0	0-150	/μL

T4

Test Requested	Result	Reference Interval	Visual Ref. Interval
T4 ⓘ	2.2	0.8-3.5	μg/dL

Urinalysis-Complete

Test Requested	Result	Reference Interval	Visual Ref. Interval
Collection Method	Not Stated		
Color	Dark Yellow		
Appearance	TURBID		
Specific Gravity	1.060	1.015-1.050	
pH	6.5	5.5-7.0	
Protein ⓘ	3+	NEGATIVE	
Glucose-Strip ⓘ	NEGATIVE	NEGATIVE	
Ketones ⓘ	NEGATIVE	NEGATIVE	
Bilirubin	2+	NEG TO 1+	
Occult Blood	2+	NEGATIVE	
WBC	0-1	0-3	HPF
RBC	11-20	0-3	HPF
Casts	NONE SEEN	Hyaline 0-3	LPF
Calcium Oxalate Dihydrate Crystals	0-1 HPF		
Amorphous Crystals	0-1 HPF		
Bacteria	NONE SEEN	None Seen	HPF
Squamous Epithelia	0-1	0-3	HPF
Fat Droplets	>50 HPF		

KeyScreen GI Parasite PCR Panel

Test Requested	Result	Reference Interval	Visual Ref. Interval
Ancylostoma spp.	Undetected		
A. caninum resistance marker	Undetected		
Uncinaria stenocephala	Undetected		
Toxocara spp.	Undetected		
Toxocara canis	Undetected		
Toxocara cati	Undetected		
Toxascaris leonina	Undetected		
Baylisascaris procyonis	Undetected		
Trichuris spp.	Undetected		
Giardia duodenalis	Undetected		
Giardia Zoonotic	Undetected		
Cryptosporidium canis	Undetected		
Cryptosporidium felis	Undetected		
Cystoisospora spp.	Undetected		
Eimeria spp.	Undetected		
Dipylidium caninum	Undetected		

Echinococcus multilocularis	Undetected
Echinococcus granulosus	Undetected
Taenia spp.	Undetected
Neospora caninum	Undetected

Comment ⓘ

Comments

SuperChem

1. PrecisionPSL – In dogs with appropriate clinical signs, this mildly elevated PrecisionPSL result is equivocal for a diagnosis of pancreatitis. In dogs without clinical signs of pancreatitis, a mild PrecisionPSL elevation is an insignificant finding.

T4

2. T4 –

Diagnosis of canine hypothyroidism: for dogs with consistent clinical signs and without non-thyroidal illness, a low-to-low normal T4 concentration (e.g., <1.0 ug/dL) could support hypothyroidism. Assessment of free T4 by ED, TSH and possibly thyroglobulin autoantibodies (TGAA) can aid in the diagnosis.

Canine hypothyroidism and synthetic T4 titration: Sample at 4 to 6 hours after dosing and aim for T4 concentrations in the mid to just above the upper end of the reference interval. Dose reduction should be considered when T4 concentration is greater than approximately 6.0 ug/dL. Always interpret results alongside the patient's clinical signs and response to treatment.

Urinalysis-Complete

3. Protein – Urine protein:creatinine ratio testing is recommended (if the sediment is inactive) to help determine the clinical significance of proteinuria.
4. Glucose-Strip – Result Verified
5. Ketones – Result Verified

KeyScreen GI Parasite PCR Panel

6. Comment –

A DETECTED KeyScreen GI Parasite PCR result in a patient with clinical signs that are appropriate to the organism, suggests this is the likely cause of the clinical signs. In the absence of clinical signs, parasite detection could suggest a subclinical infection or be related to coprophagia. Subclinical infection may need to be treated in cases where the parasite is zoonotic, has the potential to cause clinical signs or where continued shedding contributes to environmental contamination.

An UNDETECTED KeyScreen GI Parasite PCR result indicates that no parasitic organism was detected. An undetected PCR result most often indicates absence of infection but might also occur after successful treatment or with spontaneous resolution of infection. Undetected results due to cyclical shedding may be overcome with repeat testing or by testing pooled samples collected over multiple days.

For infections with an extra-intestinal phase (e.g., echinococcosis, toxoplasmosis, neosporosis), an undetected KeyScreen GI Parasite PCR result does not rule out systemic infection. If systemic infection is suspected, additional diagnostic investigation is indicated.

As a reference, we have provided links to CAPC guidelines. CAPC is an independent, non-profit organization.

Veterinarians: If the KeyScreen GI Parasite PCR result does not explain the clinical signs or if you require additional interpretive assistance, consultation with an internist is available free of charge (Monday to Friday 8am to 8pm EST, Saturday 9am to 6pm EST) at 1-888-838-4636.